CLINICAL TRIAL FAILURES · DISCONTINUATION RATE · ONCOLOGY PHASE 2,3 · STARTS 2015–2024

PARP + PD-(L)1: 3 molecules stopped early, all small molecules

5 stopped trials, but 3 molecules — all small molecules: Talazoparib, Niraparib and Rucaparib. 2 of the stops were the trial's own verdict and 1 followed a decision taken elsewhere; 2 cannot be established. Against 5.2% for PD-(L)1 combinations without PARP and 4.5% across all oncology Phase 2,3 trials in the same window. The rate counts trials that stopped early for efficacy, safety or benefit–risk reasons. Trials that ran to completion and missed their primary endpoint are counted separately below, never in the rate.

7.8%
5 of 64 closed trials (95% CI 3.4%–17.0%)

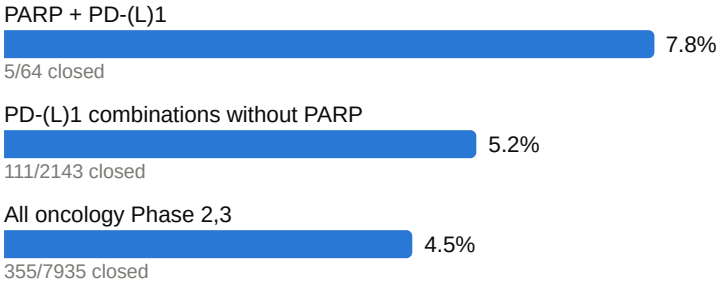
5 / 0 / 0
efficacy / safety / both (adds to 5)

33.0%
of 194 trials have closed · 130 still open or unresolved

2.6%
stopped for a biological reason within 3 years of starting (95% CI 1.0%–6.9%; 123 still at risk; 1.5% area-wide)

4 of 6 **Completed and missed the primary endpoint.** Of 43 completed trials in this segment, 6 posted a primary result on ClinicalTrials.gov that can be read — the sponsor's own comparison held to the sponsor's own threshold, or its statement that the endpoint was missed: 4 missed and 0 met, 2 split across co-primary endpoints. Counted separately and never in the rate above; a floor, not a rate. 8 of the 39 completed trials that finished more than a year ago have posted no results at all — a gap in what can be known, not a sign of failure.

How this compares



What is in this class: 195 distinct experimental drugs across 194 trials (small molecule 192, antibody 181, unknown 21, adc 13). Most tested: OLAPARIB (94), DURVALUMAB (45), PEMBROLIZUMAB (43), NIRAPARIB (39), DOSTARLIMAB (24). A class groups drugs by what they act on; check that the grouping is one you would make before reading the rate as a property of the mechanism.

How much does this rest on one decision? The 5 stops came from 5 sponsor-asset programmes across 3 sponsors; the largest (University of Chicago) contributed 1; removing that programme's trials from both sides leaves 6.3%. Screened alongside 85 other segments, the rate is within what screening that many classes could produce (q=1); the molecules and trials that failed here are the record either way. A further 2 closed trials here were terminated with no cause recorded in the registry; if every one of them were biological the rate would be 10.9%. Read it as a screen worth checking against the underlying trials.

Bars show the share of closed trials stopped for biological reasons. Confidence intervals overlap where sample sizes are small — read the counts, not just the bars.

By start cohort

Trials started	Stops	Closed	Rate	95% CI
2015–2017	0	10	0.0%	0.0%–27.8%
2018–2020	5	42	11.9%	5.2%–25.0%
2021–2024	0	12	0.0%	0.0%–24.3%

Where the stops sit

Efficacy / futility: 5.
Sponsors with most stops: Pfizer (3), GSK (1), University of Chicago (1).

The stopped trials

Trial	Ph	Sponsor	Experimental drugs	Stopped	Type	Registry stop reason
NCT03642132	3	Pfizer PFE	AVELUMAB, TALAZOPARIB, Chemotherapy followed by talazopa…	2021-12	Efficacy	Phase 3 B9991010 study was stopped due to futility of efficacy at interim analysis on 21Dec2018, and approvals of PARP inhibitors in front-line maintenance setting, Pfizer decided stopping enrollment in the B9991030 study on 19Mar2019.
NCT03955471	2	GSK GSK	NIRAPARIB, DOSTARLIMAB	2021-08	Efficacy	The study will not resume based on the results of a planned interim analysis that showed futility
NCT03637491	2	Pfizer PFE	AVELUMAB, BINIMETINIB, TALAZOPARIB	2020-12	Efficacy	Available clinical data has shown limited anti-tumor activity and reaching target study drug dose levels may not be feasible.
NCT04052204	2	Pfizer PFE	AVELUMAB, BEMPEGALDESLEUKIN, TALAZOPARIB, ENZALUTAMIDE	2020-09	Efficacy	Study prematurely terminated as the experimental treatments evaluated in study B9991040 may not provide additional clinical benefit over current or future Standard of Care in the different therapeutic indications that this trial was to evaluate.
NCT03572478	2	University of Chicago	RUCAPARIB, NIVOLUMAB	2020-02	Efficacy	The study was terminated due to lack of efficacy.

Method
Denominator: ClinicalTrials.gov interventional Phase 2,3 oncology trials started 2015–2024 that have closed (completed or terminated). Numerator: terminated trials whose registry stop reason is classified as biological (efficacy, safety or benefit–risk) by Classification V2 — held-out precision 95.5%, recall 95.3% (n=600). Drugs are linked to ChEMBL and the NCI Thesaurus; 76% of industry oncology trials in this window carry a resolved drug target, and a trial without one cannot enter a mechanism class. Intervals are Wilson 95%.

Limits
Not a failure rate: completed trials that missed their endpoints are not in the rate. They are counted separately where the sponsor posted a primary result that can be read — most did not, so that count is a floor — and programmes discontinued after a completed trial do not appear. Stop reasons are sponsor-reported. This is a closed-trial proportion, not a time-to-event analysis: only 33.0% of trials in this segment have closed, and a trial that stops early enters the denominator sooner than one that runs to completion, which can inflate the rate in immature segments. Recent cohorts have fewer closed trials, so their rates are less stable. Research signals, not clinical or investment advice.

The whole cohort, trial by trial. The diligence report names every trial behind these figures — the ones still running and the ones that completed and missed their primary endpoint included — and places your molecule against each one that failed. Free during the beta, in exchange for a work email, delivered immediately: **clinicaltrialfailures.com/asset-check**

Sources: ClinicalTrials.gov (NLM); ChEMBL (EMBL-EBI, CC BY-SA 3.0); NCI Thesaurus (NCI); RxNorm/RxClass (NLM); SEC EDGAR. Classification, linkage and rates by Clinical Trial Failures. Rebuilt weekly; this brief covers trials started 2015–2024.